S.C.O.P.E Business Case and Pilot Plan

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Executive summary

S.C.O.P.E roughly doubles the number of high-risk people reached by preventive outreach in Abu Dhabi, with the same outreach team, for an estimated AED 2 per covered resident per year.

Today, invitations to Well-Child Visits, immunisation and IFHAS screening follow age and sex rules. S.C.O.P.E adds the DoH standards, calibrated risk, family links and consented genomic results, then plans one Family Health Day per household and invites the family in its own language on WhatsApp.

In a simulation on a synthetic Abu Dhabi population, at an equal budget of 600 people contacted per 90-day cycle, S.C.O.P.E reached 74 high-risk people against 36 for age and sex rules, and found 16 undiagnosed diabetes cases against 7. With the tailoring effects halved, the gain is still about 1.7 times.

The ask: a 6-month pilot in four primary-care catchments (about 60,000 residents) with DoH/ADPHC, linked to Malaffi, with Daman as the second payer. The licence is waived for the pilot; success is judged on completed visits and calibration on real data.

The problem in numbers

Abu Dhabi already holds the data to predict preventive need, but outreach still runs on age and sex rules, and most of the hidden risk sits outside them.

Indicator	Value	Source
Abu Dhabi population, 2024	4,135,985 residents, up 7.5% in a year	SCAD
Adults 20–79 with diabetes, UAE	20.7% (age-adjusted)	IDF Diabetes Atlas 2024
Share of those cases undiagnosed	64%	IDF Diabetes Atlas 2024
Health spending per person with diabetes	USD 1,583.5 a year	IDF Diabetes Atlas 2024
Malaffi health information exchange	12.7 million patient profiles, 3,072 facilities, 3.5 billion records (Aug 2025)	M42
IFHAS adult screening	Thiqa holders aged 18 to 75 only	ADPHC

Three gaps follow from this. Children and adults with elevated risk are invited at the same time as everyone else, or not at all. Families receive separate invitations for each member, so each visit competes with work and school. Expatriate adults outside Thiqa have no funded comprehensive screening pathway; in the synthetic population that is 942 of 2,711 adults.

Value by stakeholder

Each party gets a measurable result it is already accountable for, so S.C.O.P.E can be paid from existing programme budgets rather than a new line.

Stakeholder	What they get	Measure they already report
DoH and ADPHC (programme owner)	More completed Well-Child Visits, M-CHAT-R screens, immunisations and IFHAS packages from the same outreach effort; an equity audit by segment; a FHIR feed into the Population Health Intelligence work	WCV attendance by age, M-CHAT-R uptake (target 100%), IFHAS completion
Daman / Thiqa (payer)	Earlier detection of diabetes and familial hypercholesterolaemia among members; fewer duplicate screens through claims-aware invitations	Screening completion, avoidable admissions, cost per member
Providers (SEHA/PureHealth, M42, private clinics)	Bundled family visits that fill clinic slots predictably; a ready worklist per clinician	No-show rate, slot utilisation, KPI compliance
Families	One visit for several members, at a time that fits school, work and Ramadan, in their own language, with clear data choices	Satisfaction, opt-out rate
Employers and insurers of expatriates	A route to community screening for workers with no funded pathway	Screening coverage of insured workers

Business model and pricing

S.C.O.P.E is licensed per covered resident per year, with part of the fee tied to outcomes, so the payer pays more only when more preventive care is actually delivered.

Component	Proposed price	Paid by	Notes
Platform licence	AED 2 per covered resident per year	DoH/ADPHC	Rules, risk, family planning, equity audit, FHIR API, console; about AED 8.3 million a year at full emirate scale (4.14 million residents)
Outcome fee	AED 10 per verified completed preventive action above the pre-launch baseline	Daman / Thiqa, or the programme owner	Counted from claims and Malaffi records; capped per year
Messaging	Pass-through at Meta's rate	Programme owner	UAE utility template USD 0.0157 per message (SleekFlow, Meta rates); about USD 70 a year per 10,000 residents
Expatriate community screening	Per screened worker, priced with the insurer or employer	Insurers, employers	Uses the mobile-clinic routing already in the prototype
Pilot	Licence waived	—	Implementation funded from the award and the DoH pilot pathway

These prices are working assumptions for negotiation, not quotes. They sit far below the per-person cost of the conditions found: a year of diabetes care costs USD 1,583.5 per person (IDF).

Cost and return

For every 10,000 residents, a licence of AED 20,000 a year buys about 33 extra undiagnosed diabetes cases found and 143 extra high-risk people reached, because the outreach team stays the same size and only its targeting changes.

Extra outcome per 10,000 residents per year	Base case	Tailoring effects halved	Licence cost per extra outcome (base / halved)
High-risk people reached	143	91	AED 140 / AED 219
Care gaps closed	1,157	467	AED 17 / AED 43
Autism screens (M-CHAT-R) completed	112	69	AED 179 / AED 288
Children brought up to date on vaccines	85	27	AED 234 / AED 742
Undiagnosed diabetes found	33	19	AED 602 / AED 1,042

Finding one undiagnosed diabetes case costs about AED 602 in licence fees, roughly a tenth of the USD 1,583.5 (about AED 5,815) spent each year on a person living with diabetes (IDF). We do not claim that figure as a saving; the case for value is earlier treatment at a low cost per case found.

Family genetic follow-up adds value not counted above. A 2024 review found 84% of studies judged familial hypercholesterolaemia screening potentially cost-effective, with cascade screening at a pooled USD 49,630 per QALY (Global Health Research and Policy).

How the numbers were built. Simulated gains at equal outreach capacity (600 people per 90-day cycle on 5,416 synthetic residents, 200 runs), scaled to 10,000 residents and four cycles a year, then halved to allow for gaps being used up over repeated cycles. All results come from synthetic data; the pilot replaces them with measured values.

Competitive landscape

S.C.O.P.E is built to complement the DoH–Microsoft Population Health Intelligence framework rather than compete with it: it is the family-level workflow that turns a prediction into a completed visit.

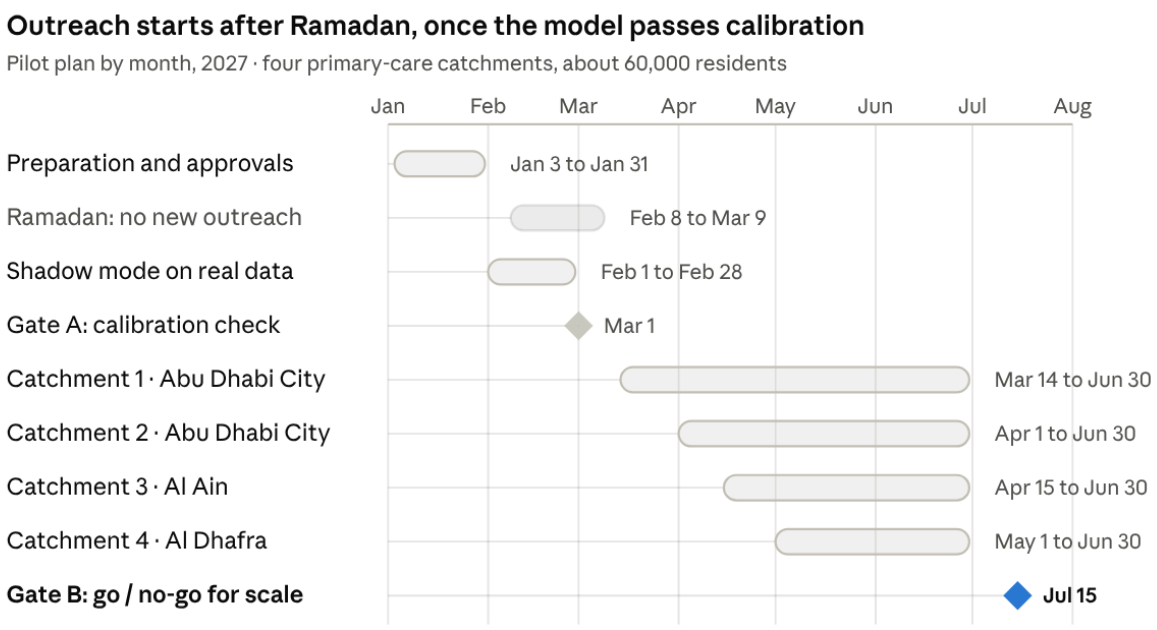
The framework signed in April 2025 has two pillars, Predict and Prevent and Act, and targets chronic disease prevention, early detection and health equity (DoH). The announcement mentions neither family-level planning nor genomics, which is where S.C.O.P.E adds most.

Capability	Age/sex reminder rules (today)	Generic population health platforms	S.C.O.P.E
Eligibility from DoH standards (WCV, immunisation, IFHAS)	Partly	Configured per client	Built in, with source on every flag
Unit of outreach	Individual	Individual	Household: one Family Health Day
Consented genomic cascade (FH, HBOC, Lynch)	No	Rarely	Yes, index-mediated, no disclosure
Arabic and five South Asian and Filipino languages	Arabic, English	Usually English-first	Seven languages, voice notes
Equity floor and small-cell suppression	No	Reporting only	Allocation and audit
FHIR R4 interface for Malaffi	Varies	Varies	Yes, with audit trail

The middle column is our assessment of typical products, not a review of named vendors.

Pilot plan

A 6-month stepped-wedge pilot in four primary-care catchments starts with a month of scoring real data without contacting anyone, then switches one catchment on at a time, so each catchment is its own comparison before activation.



The schedule keeps the first outreach clear of Ramadan and starts on 14 March, when the post-Eid relaunch window opens. Catchments not yet activated keep current practice and serve as the comparison.

Partners sought: ADPHC primary care (programme owner), Malaffi (data), Daman (claims and second payer), one provider group for the Family Health Day clinics, and a messaging partner with a verified WhatsApp Business sender.

Success criterion (proposed)	Threshold at Gate B
Calibration on real data, per major segment	Predicted/observed ratio within 0.8–1.25, or 95% interval including 1
Completed Well-Child Visits at scheduled age	Higher than in not-yet-activated catchments
M-CHAT-R uptake at 18 and 24 months	At least 90% (DoH target 100%)
IFHAS completion among invited Thiqa adults	Higher than the pre-launch baseline
Positive M-CHAT-R referred within the WCV timelines	100%
Opt-out rate after invitation	Below 5%
Privacy or security incidents	None
Family satisfaction with the Family Health Day	At least 4 out of 5

Scale path

After the pilot, S.C.O.P.E scales emirate-wide on existing infrastructure, then to other health systems that share its starting conditions.

1. **Abu Dhabi, full scale (year 2).** All primary-care catchments, through Malaffi and the FHIR API. A full nightly refresh of 3.8 million people takes under a minute on one processor core in our benchmark, so compute is not the constraint; clinic capacity and staff training are.
2. **Other emirates (year 3).** The rule base swaps per regulator (DHA, MOHAP); the family, consent and equity layers stay the same.
3. **GCC (years 3–4).** Systems with national genomic programmes, high consanguinity and multilingual expatriate populations face the same problem.
4. **Low- and middle-income settings.** The WhatsApp-first design, voice notes and community-clinic routing already work without a national data exchange. A lighter version can run on vaccination and antenatal registers, for example in sub-Saharan Africa.

Risks and mitigations

The largest risk is that real-data performance falls short of the simulation; the pilot is designed to find that out early and cheaply.

Risk	Effect	Mitigation
Model underperforms on real Malaffi data	Wrong people prioritised	One month in shadow mode before any outreach; per-segment recalibration; go/no-go gate on calibration
Tailoring effects smaller than assumed	Smaller gains	Business case still holds at half effects (about 1.7 times); effects measured in the pilot, not assumed
Data-sharing approval takes longer	Pilot start slips	Start with data the programme already holds (WCV, immunisation, IFHAS claims); add genomics once approved
Clinic capacity for bundled visits	Families booked, slots missing	Slot planner capped per clinic per day; providers co-design the Family Health Day
Message fatigue or distrust	Opt-outs rise	Verified sender, no health detail in messages, four reply options, opt-out monitored weekly
Translation quality	Misunderstood invitations	Native-speaker and clinical review before launch; family feedback in the first cycle
Equity harm in a segment	Some groups under-served	Equity floor by language and nationality; audit before each cycle; small-cell suppression

Team and clinical leadership

The project is led by a medical biologist with a PhD in public health, head of a health sciences department, who leads clinical validation of the rule base and the pilot evaluation.

Sources

- SCAD: Abu Dhabi population 2024
- IDF Diabetes Atlas: United Arab Emirates
- M42: Malaffi reaches 3.5 billion clinical records

- ADPHC: IFHAS programme
- DoH: Population Health Intelligence framework with Microsoft
- SleekFlow: WhatsApp Business pricing in the UAE
- Global Health Research and Policy: cost-effectiveness of FH screening
- UAE government portal: health insurance and Thiqa
- DoH: Standard for Well-Child Visits v10